

ECHS Forensic Fraud & Policy Abuse Explanations

This document provides a detailed breakdown of the 10 forensic patterns identified in the ECHS Module 19 audit. It explains the exact mechanism of the fraud, how ECHS policies are bypassed, and how beneficiary entitlements are misused by corrupt facilities.

Pattern 1: High Deduction (Systemic Overbilling)

- **Fraud Mechanism:** Hospitals systematically inflate the costs of diagnostic tests, pharmacy consumables, and routine packages far beyond the mandated CGHS rates.
- **Policy Abuse:** This is a "throw everything at the wall and see what sticks" strategy. Hospitals deliberately submit inflated bills knowing the BPA (Bill Processing Agency) will reject parts of it. The fraud lies in the sheer volume; if the automated checks miss even 5% of the inflated items, the hospital makes illicit profit.
- **Why it's flagged:** We isolated claims where the final billed amount was > ₹10,000 and the auditors had to deduct **more than 50%** of the bill.

Pattern 2: Room Upgrade Fraud (Entitlement Misuse)

- **Fraud Mechanism:** A patient's ECHS card entitles them to a General Ward bed, but the hospital purposefully bills ECHS for a Semi-Private or Private Ward.
- **Entitlement Misuse:** The hospital artificially upgrades the patient's room category without clinical justification. Because surgical and procedural package rates are often tied to the room category (Private ward surgeries pay more than General ward surgeries), the hospital extracts a massively higher total payout from the government for the exact same treatment.

Pattern 3: Ghost Admissions (IPD Paper Fraud)

- **Fraud Mechanism:** Claims are generated for In-Patient (IPD) hospital stays, but there is absolutely no `hospital_id` attached to the claim in the central registry.
- **Policy Abuse:** While direct reimbursements for OPD medicines often lack a hospital ID, an **In-Patient** admission physically requires a registered hospital bed. An IPD claim without a hospital ID strongly indicates a "paper-only" admission—where fake

bills are generated for a hospitalization that never physically occurred, or it was done by a completely unvetted, unregistered shadow clinic.

Pattern 4: Y-Flag Bypass (Non-Empanelled Abuse)

- **Fraud Mechanism:** Non-empanelled hospitals (hospitals not officially partnered with ECHS) are processing massive, routine medical claims.
- **Policy Abuse:** ECHS policy strictly states that non-empanelled hospitals can only be used in **life-threatening emergencies**. Here, non-empanelled clinics are bypassing this rule to process non-emergency, planned procedures (often > ₹50,000), siphoning funds away from quality-controlled partner hospitals into unvetted private hands.

Pattern 5: U-Flag Anomaly (Blind Spot Exploitation)

- **Fraud Mechanism:** Claims are successfully passing through the payment gateway despite the hospital's empanelment status being officially marked as 'U' (Unverified/Unknown).
- **Policy Abuse:** This is a technical exploitation of the BPA portal. Fraudulent actors push claims through system blind spots where validation checks fail to halt payments to unregistered entities.

Pattern 6: IPD Reversal (OPD Disguised as IPD)

- **Fraud Mechanism:** Hospitals take patients who only need minor OPD consultations or simple day-care tests and artificially admit them overnight.
- **Entitlement Misuse:** IPD packages yield significantly higher payouts than OPD visits. By forcing a patient to stay overnight unnecessarily, the hospital abuses the patient's time and drains ECHS funds to claim daily room rent and IPD-level procedural fees.

Pattern 7: Unlisted Procedures (Bypassing Price Caps)

- **Fraud Mechanism:** Hospitals bill for medical treatments using custom, vague, or non-standard procedure codes that do not exist in the official CGHS rate list.
- **Policy Abuse:** The CGHS rate list places strict price caps on specific treatments. By using "Unlisted" codes, hospitals bypass these fixed caps and arbitrarily assign massive price tags to basic procedures, forcing ECHS to negotiate or pay blindly.

Pattern 8: The Bait & Switch (Prior Approval Inflation)

- **Fraud Mechanism:** The hospital submits a highly attractive, low "Approximate Cost" estimate to the ECHS polyclinic to easily secure Prior Approval for a surgery. However, once the patient is treated, the final submitted bill is inflated by an absurd margin (e.g., > 50% higher than the estimate).
- **Policy Abuse:** This misuses the trust of the prior-approval gateway. Once the hospital locks the patient in, they aggressively upsell unneeded consumables or fabricate surgical complications post-facto to drastically inflate the final payout.

Pattern 9: Stay Extension Farming

- **Fraud Mechanism:** Patients are kept admitted in the hospital bed far beyond the standard medical recovery time for their specific ailment.
- **Policy Abuse:** Hospitals use patients as placeholders to "farm" daily room rent, daily ICU charges, and daily doctor visit fees, maximizing the revenue extracted per patient at the cost of blocking beds for actual emergencies.

Pattern 10: Emergency Gateway Bypass

- **Fraud Mechanism:** Hospitals falsely classify highly expensive, planned surgeries (e.g., routine knee replacements or cataract surgeries) as "Life-Threatening Emergencies".
- **Policy Abuse:** Standard planned procedures require stringent "Prior Approval" scrutiny from ECHS doctors. The "Emergency Gateway" skips this scrutiny to save lives. Corrupt hospitals abuse this gateway by tagging massive ₹1 Lakh+ planned bills as emergencies, avoiding upfront checks and forcing ECHS to pay the bill post-facto.